

Policy on Safety Culture, Leadership and Resources

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» promotes a culture of safety through safety-promoting behaviours, leadership awareness and involvement, adequate resources, performance targets, and use of staff feedback to improve patient safety and the quality improvement programme.

1. Purpose

The purpose of this policy is to establish the approach for safety culture, leadership awareness and involvement, resource availability, performance improvement targets and use of staff feedback.

This policy does not cover matters outside these areas.

2. Scope

This policy applies to management, leaders at all levels, departmental leaders, staff and personnel involved in patient safety and the quality improvement programme at «Hospital Name».

3. Policy standards

3.1 Safety culture

The management needs to ensure the adoption of behaviours that promote patient safety.

Key features required for a culture of safety include sharing information, reporting occurrences of incidents, learning from safety incident analysis, blame-free culture and encouragement of collaboration across disciplines and departments.

The key components of patient safety culture are informed culture, reporting culture, learning culture, just culture and flexible culture.

The management needs to measure its safety culture regularly, at least once a year.

This shall be measured using validated surveys.

Examples include the Manchester Patient Safety Framework (MaPSaF), Safety Attitudes Questionnaire and AHRQ Surveys on Patient Safety Culture (SOPS).

The management shall act on its patient safety culture assessment results.

3.2 Leadership awareness

The organisation and departmental leaders are aware of the patient safety and quality improvement programme, its intent and applicability to their respective areas and how it contributes to the organisation as a whole.

3.3 Departmental leadership involvement

Departmental leaders are involved in patient safety and quality improvement.

Each department could have department objectives or key performance indicators including clinical indicators.

The responsibility of achieving them could be that of the leader or designated person.

This could be discussed during departmental meetings.

3.4 Resources

Appropriate fund allocation is done by the organisation for the smooth functioning of the patient safety and quality improvement programme.

The budget could be earmarked based on previous year's spending.

If no data is available, the organisation could make a beginning by earmarking a budget but reviewing it at the end of six months to make any necessary modifications.

The management makes available adequate resources required for the patient safety and quality improvement programme.

Resources shall include men, material, machine, money, milieu, measurement and method.

These shall be in steady supply to ensure that the programme functions smoothly.

3.5 Organisational performance improvement targets

The management shall identify the organisation and department level quality objectives, set targets, monitor them at least once in three months and modify the target at least annually.

The targets shall be shared with the faculty and staff and regular feedback taken.

3.6 Workforce feedback

The feedback shall be obtained from the staff on their understanding and use of the safety and quality systems.

Feedback may be obtained once a year through staff surveys.

It is also important that staff workforce feel able to raise concerns whenever they occur.

These inputs shall be used to improve patient safety and quality improvement programmes.

4. Non-negotiable rules

1. Do not measure safety culture less frequently than at least once a year.
2. Do not measure safety culture without using a validated survey.
3. Do not leave patient safety culture assessment results without action by management.
4. Do not omit any of the seven required resource categories: men, material, machine, money, milieu, measurement and method, when providing resources for the patient safety and quality improvement programme.
5. Do not allow the required resources to fall outside the steady supply required for the programme to function smoothly.
6. Do not monitor organisational and departmental performance improvement targets less frequently than at least once in three months.
7. Do not leave performance improvement targets unmodified beyond the required annual modification.
8. Do not withhold organisational and departmental performance improvement targets from faculty and staff.
9. Do not fail to obtain staff feedback on their understanding and use of the safety and quality systems.
10. Do not fail to use the staff feedback obtained to improve patient safety and quality improvement programmes.

5. What we do

5.1 Build and measure safety culture

Management ensures adoption of behaviours that promote patient safety.

Information sharing, incident reporting, learning from safety incident analysis, a blame-free culture and collaboration across disciplines and departments support the culture of safety.

The target culture is described through informed culture, reporting culture, learning culture, just culture and flexible culture.

Safety culture is measured at least once a year using a validated survey.

MaPSaF, Safety Attitudes Questionnaire and AHRQ Surveys on Patient Safety Culture (SOPS) are examples of validated survey instruments; the organisation selects its validated survey.

Management acts on the patient safety culture assessment results.

5.2 Maintain leadership awareness

Organisation and departmental leaders are aware of the patient safety and quality improvement programme, its intent, applicability to their areas and contribution to the organisation as a whole.

5.3 Involve departmental leaders

Departmental leaders are involved in patient safety and quality improvement.

Departments could use objectives or key performance indicators including clinical indicators, with responsibility assigned to a leader or designated person and discussion during departmental meetings.

5.4 Provide adequate resources

The organisation makes appropriate fund allocation for the smooth functioning of the programme.

Management makes adequate resources available.

The resources shall include men, material, machine, money, milieu, measurement and method, in steady supply.

Budgeting based on previous-year spending and a six-month review where no data is available could be used.

5.5 Set and monitor improvement targets

Management identifies organisation- and department-level quality objectives and sets targets.

Targets are monitored at least once in three months and modified at least annually.

Targets are shared with faculty and staff and regular feedback is taken.

5.6 Obtain and use staff feedback

Staff feedback is obtained on understanding and use of safety and quality systems.

An annual staff survey may be used to obtain feedback.

Staff should feel able to raise concerns whenever they occur.

Inputs obtained are used to improve patient safety and quality improvement programmes.

6. Governance and responsibility

Role	Responsibility
Management	Ensures safety culture measurement and action on results, makes adequate resources available, and identifies, monitors and modifies performance improvement targets.
Organisation and departmental leaders	Leaders are aware of the patient safety and quality improvement programme, its intent, applicability and contribution to the organisation. Departmental leaders are involved in patient safety and quality improvement.
Staff	Faculty and staff receive organisational and departmental performance improvement targets and provide regular feedback. Staff feedback on understanding and use of safety and quality systems is obtained.
Responsible personnel	Responsible personnel support implementation of the patient safety and quality improvement programme and use feedback to improve it.

7. Quality monitoring

Monitoring area	What is monitored
Safety culture	Measurement using a validated survey at least once a year and action on results
Leadership awareness	Awareness of programme intent, applicability and contribution to the organisation
Departmental leadership	Involvement in patient safety and quality improvement
Resources	Men, material, machine, money, milieu, measurement and method in steady supply
Performance targets	Organisation and department objectives and targets; monitoring at least once in three months and modification at least annually
Target communication	Targets shared with faculty and staff and regular feedback taken
Staff feedback	Feedback on understanding and use of safety and quality systems and its use for improvement

8. Training and staff acknowledgement

Staff involved in patient safety and the quality improvement programme are familiar with the processes described in this policy.

I have read the Policy on Safety Culture, Leadership and Resources of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to management, leaders, departmental leaders and staff involved in patient safety and the quality improvement programme.

10. Abbreviations

Abbreviation	Meaning
MaPSaF	Manchester Patient Safety Framework
AHRQ	Agency for Healthcare Research and Quality
SOPS	Surveys on Patient Safety Culture

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
PSQ.6.a	Achievement	Sections 3.1 and 5.1 address safety-promoting behaviours, annual validated-survey measurement and action on assessment results.
PSQ.6.b	Commitment	Sections 3.2 and 5.2 address awareness among organisation and departmental leaders of the programme, its intent, applicability and contribution.
PSQ.6.c	Commitment	Sections 3.3 and 5.3 address departmental leader involvement while preserving the illustrative nature of KPIs, designated responsibility and

		departmental meetings.
PSQ.6.d	Commitment	Sections 3.4 and 5.4 address adequate resources and the seven required resource categories in steady supply while preserving advisory budgeting mechanisms.
PSQ.6.e	Achievement	Sections 3.5 and 5.5 address quality objectives, targets, monitoring at least once in three months, modification at least annually, sharing and feedback.
PSQ.6.f	Excellence	Sections 3.6 and 5.6 address mandatory staff feedback and its use for improvement while preserving the optional annual survey mechanism.

12. Required Records/Evidence Checklist

Safety culture

- Validated safety-culture survey used at least once a year.
- Safety-culture assessment results and management actions taken on the results.
- Materials showing safety-promoting behaviours, information sharing, incident reporting, learning from safety incident analysis, blame-free culture and collaboration across disciplines and departments.

Leadership

- Information showing organisation and departmental leaders understand the patient safety and quality improvement programme, its intent, applicability to their areas and contribution to the organisation as a whole.
- Documents showing departmental leader involvement in patient safety and quality improvement.
- Department objectives or key performance indicators, leader/designated-person responsibility or departmental meeting discussion where the organisation has chosen to use these mechanisms.

Resources

- Budget or fund-allocation information for the patient safety and quality improvement programme.
- Resources showing men, material, machine, money, milieu, measurement and method are available in steady supply.
- Previous-year spending basis or six-month budget review where the organisation has chosen to use these mechanisms.

Performance improvement targets

- Organisation-level and department-level quality objectives and targets.
- Monitoring records showing targets are monitored at least once in three months.
- Records showing targets are modified at least annually.
- Targets shared with faculty and staff and regular feedback taken.

Staff feedback

- Staff feedback on understanding and use of the safety and quality systems.
- Records showing staff feedback is used to improve patient safety and quality improvement programmes.
- Annual staff survey where the organisation has chosen to use this mechanism.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PSQ.6.
2. University of Manchester. (2006). <https://psnet.ahrq.gov/issue/manchester-patient-safety-framework-mapsaf> Manchester Patient Safety Framework (MaPSaF).
3. Sexton, J. B., Helmreich, R. L., Neilands, T. B., Rowan, K., Vella, K., Boyden, J., Roberts, P. R., & Thomas, E. J. (2006). The Safety Attitudes Questionnaire: Psychometric properties, benchmarking data, and emerging research. BMC Health Services Research, 6, 44. <https://doi.org/10.1186/1472-6963-6-44>
4. Agency for Healthcare Research and Quality. (2019). <https://www.ahrq.gov/sops/surveys/hospital/> Surveys on Patient Safety Culture (SOPS) Hospital Survey.

Disclaimer

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